To: David J. Freeland
2171 Chile Pepper Blvd.
San Chalopos, CA 93001

Re: Sample, Manuel

Date of Injury/Onset: May 1, 2007

Dear Mr. Freeland:

On May 7, 2007, Mr. Manuel Sample presented himself for an initial examination and evaluation of his complaints arising from a motor vehicle accident that he was involved in on May 1, 2007.

CASE SUMMARY NARRATIVE

ACCIDENT DESCRIPTION:

The time was 7:00pm. Mr. Sample stated that he was the driver in a SUV which was stopped at a light at approximately 0 m.p.h. According to the patient, the other vehicle involved was travelling at approximately 25-30 m.p.h. He stated that the other vehicle struck his vehicle in the rear end.

Mr. Sample also reported that, at the time of the accident, the road conditions were clean and dry. In addition, he stated the damage to his SUV was mild due to the fact that the other car hit his trailer hitch. Damage to the other vehicle was moderate. He also stated that he did not see the accident coming, and therefore was not braced for the impact. Also, he was wearing his seat belt and had his shoulder harness on. On impact, neither the driver's or the front passenger's air bag deployed.

His SUV was equipped with headrests. He also noted that he had his head facing straight forward at the moment of impact. On impact the patient's body did not strike the inside of his vehicle. He stated that he did not lose consciousness during the accident. Mr. Sample reports that he and his wife went to Seton Medical Center the following day. They were given a prescription for Ibuprofen, an analgesic, and Soma, a muscle relaxer and released. On release he was given instructions to rest and ice his injuries. The patient did not have x-rays taken following his injury.

INITIAL COMPLAINTS:

Immediately following the accident, the patient's main complaints included neck pain and stiffness, pain in the low back, fatigue, and pain in the upper back. Following the accident Mr. Sample drove himself home.

CURRENT COMPLAINTS:

Mr. Sample's current signs and symptoms were assessed today. His first symptom is sharp, aching and spastic pain in the neck bilaterally. He reported that the pain radiates into both shoulders. It occurs between three fourths and all of the time when he is awake, and causes *serious* diminution in his capacity to carry out daily activities. It is aggravated by bending forward, bending backward, bending to the left, bending to the right, twisting to the left, twisting to the right, coughing, sneezing, straining and by lifting.

Mr. Sample's second stated symptom is sharp, aching and spastic pain in the low back bilaterally. It also radiates into his buttocks. It occurs between three fourths and all of the time when he is awake, and causes *serious* diminution in his capacity to carry out daily activities. It is aggravated by bending forward, bending backward, bending to the left, bending to the right, twisting to the left, twisting to the right, coughing, sneezing, straining, standing and by lifting.

He stated his third symptom is dull, aching and spastic pain in the upper back bilaterally. It occurs between three fourths and all of the time he is awake, and is tolerated but it does cause *some* diminution in his capacity to carry out daily activities. It is aggravated by bending forward, bending backward, bending to the left, bending to the right, coughing, sneezing, straining, standing and by lifting.

HISTORY:

Mr. Sample indicated that he had not experienced prior symptoms similar to his current complaints, and was symptom free at the time of the aforementioned accident/onset of May 1, 2007.

I have determined that Mr. Sample’s history has not contributed to his present condition.

Prior Treatment Information:

The patient reported that prior to his first visit to this office, he was treated at Seton Medical Center. His first visit there was on May 2, 2007. During his visits to that office, Mr. Sample received examination and pain meds, which he reported had little, if any, benefit. The patient is no longer receiving treatments at that office.

ACTIVITIES OF DAILY LIVING ASSESSMENT:

Based on an assessment of Mr. Sample’s history, along with his subjective complaints, objective findings, and other test results, it is evident from a standpoint of medical certainty, that his current condition did result from the type of injury/onset described in this report. He reported suffering varying degrees of losses of functional capacity with the following activities:

With regard to *Self Care and Personal Hygiene*, Mr. Sample stated: making his bed can be performed, despite significant pain, but only if he has help; bathing, showering, washing his hair, drying his hair, combing his hair, washing his face, putting on his shirt, putting on his shoes, tying his shoes, putting on his pants, taking out the trash and going to the toilet can be managed by himself, despite marked pain; brushing his teeth, eating and cleaning dishes can be done without much difficulty, despite some pain.

With regard to *Physical Activity*, Mr. Sample stated: standing, standing for long periods, walking, stooping, squatting, sitting continuously, kneeling, bending forward, bending backward, bending to the left, bending to the right, walking for long periods, twisting to the left, twisting to the right, leaning forward, leaning backwards, leaning to the left, leaning to the right and kneeling for long periods can be managed alone, despite marked pain; sitting, reclining and reaching can be done, but not without some difficulty because of the resulting pain.

Regarding *Functional Activities*, Mr. Sample stated: lifting weights off the floor can be performed, despite significant pain, but only if he has help; carrying large objects, lifting weights off of a table, pushing things while standing and pulling things while standing can be managed by himself, despite marked pain; carrying small objects, climbing stairs and climbing up any type of incline can be done without much difficulty, despite some pain.

Regarding *Travel*, Mr. Sample stated: driving for long periods of time can be managed by himself, despite marked pain; driving a motor vehicle and riding as a passenger in a motor vehicle can be done without much difficulty, despite some pain.

GENERAL PHYSICAL EXAMINATION:

Mr. Sample is a right-handed 37 year-old mentally alert and cooperative male.

Date of Birth: October 1, 1969.

His superficial appearance suggested he was in distress. **Minor's Sign** was present. This sign is present when the patient, in arising from a chair, leans forward, jackknifing the thighs and the dorsolumbar spine so that the head is over the feet. Using the hands on the thighs or the arms of the chair, the patient pushes the body to an upright position, thus sparing lower limb effort. The presence of this sign is usually indicative of sciatica and or low back pain.

Gait: His walk revealed a mild antalgic gait.

Stature: Well developed.

RANGE OF MOTION STUDIES:

The following joint range of motion calculations and analyses were performed to determine Mr. Sample's present condition with regard to joint motion.

Cervical Spine:	**Angle**	**Analysis**
Flexion	65 degrees	Moderate restriction:norm is This test brought on both pain and spasms.	75 degrees.
Extension	45 degrees	Mild-moderate restriction: The above caused pain and spasms.	norm is 55 degrees.
Left Lateral Flexion	30 degrees	Mild-moderate restriction: The above caused pain and spasms.	norm is 40 degrees,
Right Lateral Flexion	30 degrees	Mild-moderate restriction: The above caused pain and spasms.	norm is 40 degrees,
Left Rotation	65 degrees	Moderate restriction: The above caused pain and spasms.	norm is 80 degrees,
Right Rotation	65 degrees	Moderate restriction: The above caused pain and spasms.	norm is 80 degrees,

Thoracolumbar Spine:	**Angle**	**Analysis**
Flexion	90 degrees	No restriction: This test brought on both pain and spasms.	norm is 90 degrees.
Extension	20 degrees	Moderate restriction: This test brought on both pain and spasms.	norm is 30 degrees.
Left Lateral Flexion	25 degrees	Mild restriction: The above caused pain and spasms.	norm is 30 degrees,
Right Lateral Flexion	25 degrees	Mild restriction: norm is The above caused pain and spasms.	30 degrees,
Left Rotation	30 degrees	No restriction: norm is 30 The above caused pain and spasms.	degrees,
Right Rotation	30 degrees	No restriction: norm is 30 The above caused pain and spasms.	degrees,

ORTHOPEDIC EVALUATION:

Cervical Lesion Tests:
The O'Donoghue Manuuver Test was positive bilaterally. A positive with this test indicates muscular strain and/or ligamentous sprain. Mr. Sample exhibits a Type II cervical acceleration-deceleration injury with associated mild ligamentous sprain, muscle strain and spasm. **The Shoulder Depression Test** was positive bilaterally for localized neck pain, which indicates adhesions of the soft tissues. Production or aggravation of radicular pain indicates adhesions of the dural sleeves, spinal nerve roots, or adjacent structures of the joint capsule of the shoulder.
The Cervical Compression Test was positive for localized pain. Localized pain indicates possible foraminal encroachment. Radicular pain indicates pressure on the nerve root.

Lumbar Lesion Tests:
Kemp's Test was positive for local pain which suggests facet irritation, or a strain/sprain.
The Belt Test was positive for a lumbar lesion.
The Straight Leg Raise Test was positive and suggests sciatica from lumbosacral or sacroiliac lesions, subluxation syndrome, disc lesions, spondylolisthesis, adhesions, or IVF occlusion.
Fajersztajn's or The Well Leg Raise Test is a Straight Leg Raise done on the unaffected side. If pain is elicited on the affected side the test is positive and a strong indicator of a possible herniated disc.This test was positive.
The Double Leg Raise Test was positive. This test is specific and highly accurate for lumbosacral joint lesions.
Nachlas' Test was positive bilaterally. This test is considered positive when the patient experiences pain in the lumbosacral region, which was the case with Mrs. Gomez, or the sacroiliac region, or in both regions.
Braggard's Sign was positive and indicates possible disc herniation, sciatic neuritis, spinal cord tumors, or spinal nerve irritation.
Ely's Sign was positive. With the patient prone, the heel of the affected leg is approximated to the opposite buttock. After flexion of the knee, the thigh can be hyperextended. With irritation of the iliopsoas or it sheath, it will be impossible to extend the thigh. Pain in the anterior thigh is a positive finding and indicates inflammation of the lumbar nerve roots, especially the upper lumbar.

Sacroiliac Lesion Tests:
Hibb's Test was positive bilaterally. This test is performed with the patient in a prone position. The examiner, while stabilizing the pelvis on the side nearest to him, flexes the opposite knee to a right angle. From this position, the examiner slowly laterally pushes the leg causing strong internal rotation of the femoral head. The test is done bilaterally. Pelvic pain reveals a positive test.
The Iliac Compression Test was positive bilaterally. This test is performed with the patient lying on the side on a padded table. The examiner with both hands over the superior innominate bone applies strong downward pressure. When this maneuver

elicits pain from the pelvis, the test is considered positive, and indicates a sacroiliac lesion or a sprain of the posterior sacroiliac ligaments.
Nachlas' Test was positive bilaterally. This test is considered positive when the patient experiences pain in the lumbosacral region, the sacroiliac region, or in both regions.
Yeoman's Test with the patient prone, pressure is applied to the SI joint. The knee if flexed and the heel is brought to the same side buttocks. The hip is extended by lifting the knee off the table. This applies pressure to the anterior sacroiliac joint. If pain is increased in the sacroiliac area this indicates a sacroiliac injury.

Sciatic Nerve Lesion Tests:
Braggard's Sign was positive and indicates possible disc herniation, sciatic neuritis, spinal cord tumors, or spinal nerve irritation.

Intervertebral Disc Syndromes:
The Bowstring Sign was present bilaterally. This test is done with the patient supine. The examiner performs straight leg raising until the patient experiences some discomfort. At this level the examiner flexes the knee slightly and rests the foot on his or her shoulder until any pain subsides. The examiner then applies pressure to the hamstrings. If this doesn't produce pain, the examiner moves the thumbs over the popliteal fossa and applies pressure over the popliteal. If pain is reproduced in the leg or in the back, this sign is considered to be present and indicates nerve root compression.
Fajersztajn's or The Well Leg Raise Test is a Straight Leg Raise done on the unaffected side. If pain is elicited on the affected side the test is positive and a strong indicator of a possible herniated disc.This test was positive.
Kemp's Test was positive for local pain which suggests facet irritation, or a strain/sprain.

Postural Evaluation:
A mild anterior gravitation of the head was noted. An mild elevation of the shoulder on the left side was noted by visual inspection. When comparing the left and right sides of the ilium, a slightly higher crest on the left side was noted. Additionally, a mild increased lumbar lordosis was noted.

ASSESSMENT/DIAGNOSIS:
It is expected that Mr. Sample will experience favorable results from his treatments.

Acute, moderate, constant, sprain/strain of the cervical spine with associated cervicalgia. Acute, moderate, constant sprain/strain of the lumbar spine with associated low back pain. Acute, slight, constant sprain/strain of the thoracic spine with associated thoracic pain. Subluxations of the cervical, thoracic, and lumbar spine and pelvis.

ICD-9 Coding: 847.0, 847.2, 847.1, 724.2, 723.1, 739.1, 739.3, 739.2, 724.1

PROGNOSIS:

At this time, Mr. Sample's prognosis is good. His case is somewhat complicated, but continued improvement is expected, despite permanent residuals being a possibility.

TREATMENT PLAN:

Chiropractic Manipulative Treatment: Chiropractic Diversified and Drop Table full-spine adjusting.

Physical Therapy: Microcurrent for tissue healing and pain control. Ice or heat as needed to reduce pain and inflammation. Therapeutic stretching and strengthening exercises to restore functional biomechanics.

Nutritional support: Nutritional supplements consisting of Omega-3 Fatty Acids, Glucosamine and Chondroitin Sulfates, and MSM to reduce pain and inflammation and to facilitate the healing processes.

.

FUTURE CARE PLAN:

Present Care Phase: Currently, we have the patient in a relief phase of care. This includes: Chiropractic adjustments, microcurrent, ice packs, and nutritional support.

Future Treatment Plan: Mr. Sample's future care plan includes home exercises, physical therapy, massage therapy, trigger point therapy, chiropractic adjustments, resistive strengthening exercises, and nutritional support.

Goals of Treatment Plan: The preceding treatment plan has the goal of decreasing pain, decreasing swelling and inflammation, decreasing spasms, increasing range of motion, increasing the ability to perform normal activities of daily living, increasing strength, returning the patient to his pre-accident status, increasing function, retarding degeneration, correcting muscle imbalance, increasing flexibility, achieving maximum chiropractic improvement, reducing frequency and severity of probable exacerbations and improving alignment.

CLOSING COMMENTS:

Mr. Sample's complaints are consistent with the type and severity of accident he was involved in.

PRIMARY TREATING PHYSICIAN:

I declare that this report is true and correct to the best of my knowledge.

Signature:

Executed at: ***Second the Motion Chiropractic*** **Date:** 05/15/07
Name: Buzz Lightyear, D.C. **Cal. Lic.#** DC70007
Address: 31001 Broadway Boulevard **Phone:(**605**)**383-3838
Los Ponchos, CA 90303

Report Submission Date:

Name:		**Date:**	
MRN:		**Modality Type:**	MR
Gender:		**Description:**	MR CERVICAL SPINE WO
DOB:		**Laterality:**	
		Referrer:	Second Opinions

MRI cervical spine without contrast.

CLINICAL HISTORY: Reason for Initial Exam Pain, heaviness in arms and hands.

TECHNIQUE: Fast spin echo T2 and spin echo T1 sequences were obtained in axial and sagittal planes.

Findings:

Previous MRIs dated 04/10/23 and 1/15/21 is available for comparison.

There is straightening of cervical lordosis.

There is mild chronic anterior wedging of C5 vertebral body.

The rest of the osseous elements are intact with no evidence of fracture or dislocation.

The cervical cord is of normal uniform signal intensity without evidence of focal expansion.

There is no evidence for tonsillar herniation. Limited views of the posterior fossa reveal no abnormalities.

Multilevel disk dehydration, reduced disc heights and marginal osteophytes are seen.

At C2-C3, there is a 10 x 1.5 mm disc herniation showing an annular tear indenting the ventral thecal sac causing minimal left neural foramina narrowing. Canal is patent.

At C3-C4, there is a 11 x 3 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing moderate left and mild-to-moderate right neural foramina narrowing. Canal is borderline stenotic.

At C4-C5, there is a 12 x 3 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing mild right and mild-to-moderate left neural foramina narrowing. Canal is mildly stenotic.

At C5-C6, there is a 11 x 2.5 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing mild right and minimal left neural foramina narrowing. Canal is borderline stenotic.

At C6-C7, there is a 12 x 3 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing mild-to-moderate right and mild left neural foramina narrowing. Canal is borderline stenotic.

Remaining levels are unremarkable.

IMPRESSION:

1. Straightening of cervical lordosis.

2. At C2-C3, there is a 10 x 1.5 mm disc herniation showing an annular tear indenting the ventral thecal sac causing minimal left neural foramina narrowing. Canal is patent.

3.At C3-C4, there is a 11 x 3 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing moderate left and mild-to-moderate right neural foramina narrowing. Canal is borderline stenotic.

4.At C4-C5, there is a 12 x 3 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing mild right and mild-to-moderate left neural foramina narrowing. Canal is mildly stenotic.

5.At C5-C6, there is a 11 x 2.5 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing mild right and minimal left neural foramina narrowing. Canal is borderline stenotic.

6.At C6-C7, there is a 12 x 3 mm disc herniation indenting the ventral thecal sac with uncovertebral hypertrophy causing mild-to-moderate right and mild left neural foramina narrowing. Canal is borderline stenotic.

7.As compared to the previous MRIs dated 04/10/23 and 1/15/21, there is evidence of small additional disc herniation at C2-C3 level with mild increase in size of the disc herniations at C3-C4 and C4-C5 levels.

SECOND OPINIONS DISCLAIMER:
Note: The above report was provided by the signing radiologist at the request of the referring physician, patient or patient representative as a second opinion. The opinions and recommendations included therein are provided on a strict advisory basis and are based on the images and clinical information provided. Neither the radiologist nor the company assume responsibility for any decisions made or actions taken based on this report nor for any effects that result from such decisions and actions. The responsibility for clinical decisions remains entirely upon the patient's medical team. This information has been clearly communicated at the time of request of this second opinion.

Electronically signed on by:

Fellowship Trained MRI and CT Specialist

USARAD Holdings Inc. / SecondOpinions.com 855-573-2663
2805 East Oakland Park Boulevard, Suite 250, Fort Lauderdale, FL, US, 33306

INTEGRITY
DIAGNOSTICS

Test Date: 1/2/2020

Patient:	Sample Radiculopathy Case	**DOB:**	7/7/1947	**Physician:**	Dr. MD
Sex:	Male				

Diagnosis: Cervical radiculopathy M54.12

Complaint:
The patient complains of lower cervical pain and paresthesia in the left shoulder and lateral arm. There are no complaints of right sided symptoms or lower extremity symptoms and past medical history is negative for DM, vitamin deficiency, excessive ETOH, thyroid or other autoimmune disorders. A brief clarifying exam is performed prior to testing and temperature is maintained above 31C in the left upper extremity.

ELECTRONEUROMYOGRAPHIC SUMMARY

Nerve Conduction:

Sensory NCS: The left median, radial and ulnar nerves are WNL.

Motor NCS: The left median, radial and ulnar nerves are WNL.

Late Responses: The left median and ulnar F-waves are WNL.

Electromyography:
Using a monopolar needle electrode, sampling consisted of the dorsal primary rami innervated muscles from the left C5-T1 paraspinals as well as ventral primary rami innervated muscles of the C5-T1 myotomes in the upper extremity. Muscle membrane instability (fibrillation/positive sharp wave activity) was observed in the left C6-7 paraspinals, the left triceps and left FCR muscles. Paraspinal assessment was technically challenging due to decreased patient ability to consistently maintain electrical silence during assessment. All other muscles tested showed normal activity at insertion, rest and during voluntary recruitment.

Impression: (Abnormal Study)

1. **Evidence suggests a mild acute left lower cervical radiculopathy (C6-7) that affects the dorsal and ventral primary rami.**

2. **There is no evidence to suggest a polyneuropathy, plexopathy or focal entrapment.**

INTEGRITY
DIAGNOSTICS
Dr. Eric C. Bull PT, ECS
Dr. Eric Bull, PT, DSc, MPT, MMT, ECS, OCS
ABPTS Board Certified Clinical Electrophysiology Specialist
Licensure: GA 7633 | SC CP025047T | FL 41592
Cellular: (912) 856-1410 | Email: Eric.Bull@Integrity-Dx.com

Nerve Conduction Studies

Anti Sensory Summary Table

Site	NR	Peak (ms)	Norm Peak (ms)	O-P* Amp (µV)	Norm O-P Amp	Site1	Site2	Delta-P (ms)	Dist (cm)	Vel (m/s)	Norm Vel (m/s)
Left Median Anti Sensory (3rd Digit)											
Palm		1.8	<2.2	39.1	>10	Palm	3rd Digit	1.8	7.0	39	
Wrist		3.1	<3.6	32.2		Wrist	Palm	1.3	7.0	54	>39
Left Ulnar Anti Sensory (5th Digit)											
Wrist		2.6	<3.4	22.6	>10	Wrist	5th Digit	2.6	12.0	46	

Motor Summary Table

Site	NR	Onset (ms)	Norm Onset (ms)	O-P* Amp (mV)	Norm O-P Amp	Neg Dur (ms)	Site1	Site2	Delta-0 (ms)	Dist (cm)	Vel (m/s)	Norm Vel (m/s)
Left Median Motor (Abd Poll Brev)												
Wrist 8cm		3.6	<4.2	4.4	>2.8	5.63	Elbow	Wrist 8cm	3.7	21.5	58	>50
Palm		1.9		4.7		5.78	Wrist 8cm	Palm	1.7	5.5	32	>29
Elbow		7.3		4.0		5.94						
Left Radial Motor (Ext Ind Prop)												
Forearm 8cm		2.7	<3.0	7.2		7.66	Brachium	Forearm 8cm	3.2	20.0	63	>50
Brachium		5.9		7.1		7.50						
Left Ulnar Motor (Abd Dig Minimi)												
Wrist 8cm		2.8	<3.6	7.3	>4.2	5.00	B Elbow	Wrist 8cm	2.8	17.0	61	>49
B Elbow		5.6		7.0		5.31	A Elbow	B Elbow	1.9	10.0	53	>47
A Elbow		7.5		6.9		5.47						

Comparison Summary Table

Site	NR	Peak (ms)	O-P* Amp (µV)	Norm O-P Amp	Site1	Site2	Delta-P (ms)
Left Median/Radial Comparison (Digit 1)							
Median 10cm		2.6	23.0	>7uV	Median 10cm	Radial 10 cm	0.1
Radial 10 cm		2.5	7.6	>5uV			

F Wave Studies

NR	F-Lat (ms)	Lat Norm (ms)	L-R F-Lat (ms)	L-R Lat Norm
Left Median (Mrkrs) (Abd Poll Brev)				
	21.71	<33		<2.0
Left Ulnar (Mrkrs) (Abd Dig Min)				
	24.42	<36		<2.0

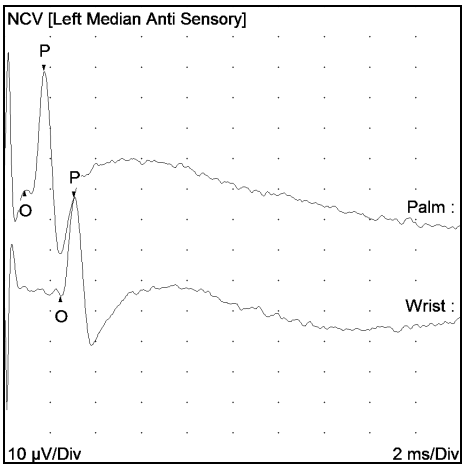
EMG

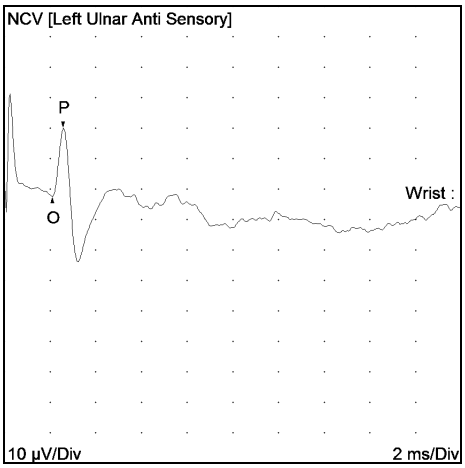
Side	Muscle	Nerve	Root	Ins Act	Fibs	Psw	Other	Amp	Dur	Poly	Recrt	Int Pat	Comment
Left	1stDorInt	Ulnar	C8-T1	Nml	Nml	Nml	Nml	Nml	Nml	0	Nml	Nml	
Left	FlexCarRad	Median	C6-7	**Incr**	**1+**	Nml	Nml	Nml	Nml	0	Nml	Nml	
Left	BrachioRad	Radial	C5-6	Nml	Nml	Nml	Nml	Nml	Nml	0	Nml	Nml	
Left	Triceps	Radial	C6-7-8	**Incr**	Nml	**1+**	Nml	Nml	Nml	0	Nml	Nml	
Left	Deltoid	Axillary	C5-6	Nml	Nml	Nml	Nml	Nml	Nml	0	Nml	Nml	
Left	ExtDigCom	Radial (Post Int)	C7-8	Nml	Nml	Nml	Nml	Nml	Nml	0	Nml	Nml	

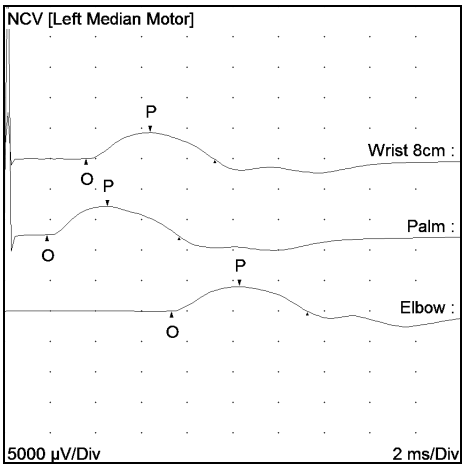
Paraspinal EMG

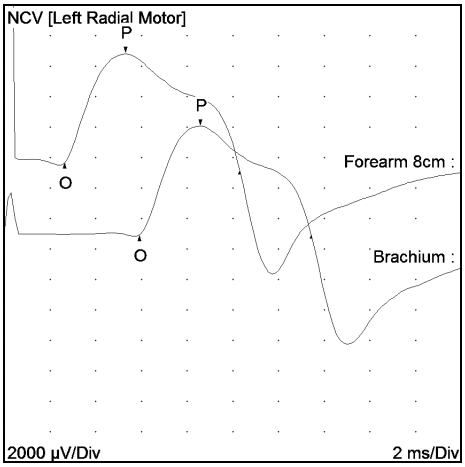
Side	Muscle	Nerve	Root	Ins Act	Fibs	Psw	Other	Amp	Dur	Poly	Recrt	Int Pat	Comment
Left	C5-6	DPR	C5-6	Nml	Nml	Nml	Nml	NT'd	NT'd	NT'd	NT'd	NT'd	
Left	C6-7	DPR	C6-7	**Incr**	**1+**	**1+**	Nml	NT'd	NT'd	NT'd	NT'd	NT'd	
Left	C7-T1	DPR	C7,C8	Nml	Nml	Nml	Nml	NT'd	NT'd	NT'd	NT'd	NT'd	

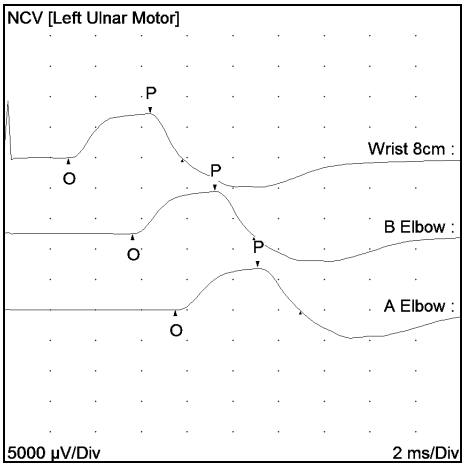
Waveforms:

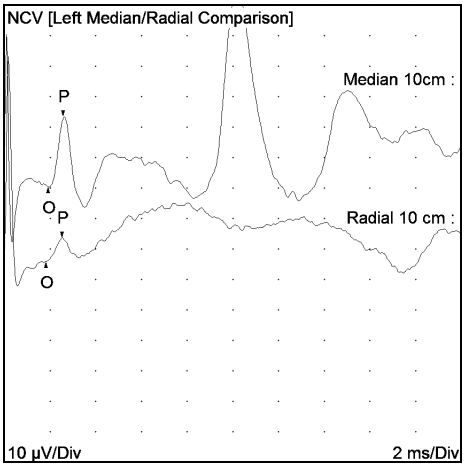

NCV [Left Median Anti Sensory]
P
O
P
Palm :
O
Wrist :
10 µV/Div
2 ms/Div

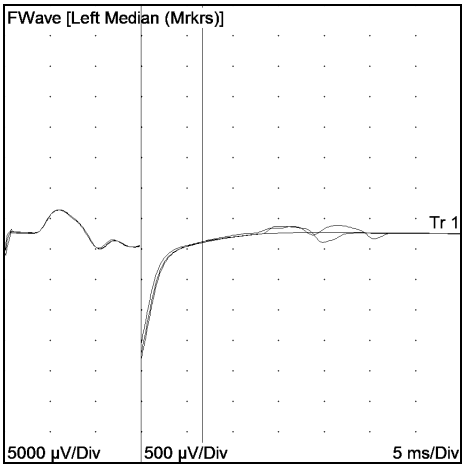

NCV [Left Ulnar Anti Sensory]
P
O
Wrist :
10 µV/Div
2 ms/Div

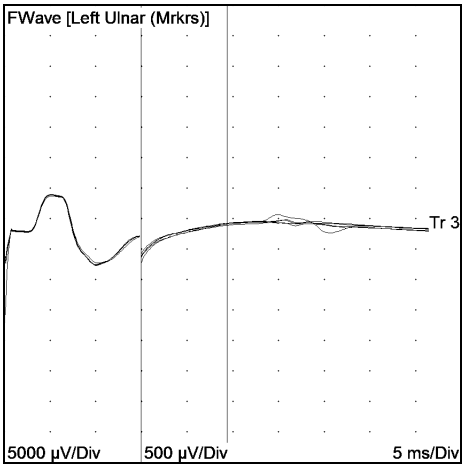

NCV [Left Median Motor]
P
Wrist 8cm :
O
P
Palm :
O
P
Elbow :
O
5000 µV/Div
2 ms/Div

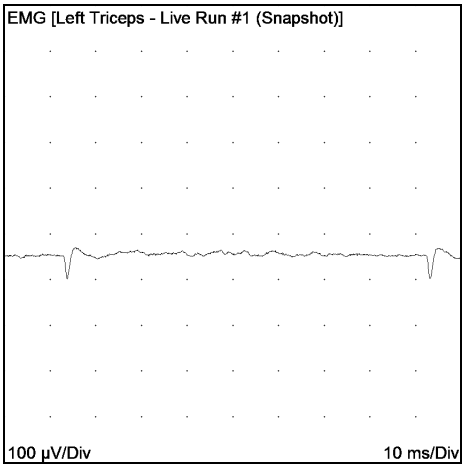

NCV [Left Radial Motor]
P
P
Forearm 8cm :
O
O
Brachium :
2000 µV/Div
2 ms/Div

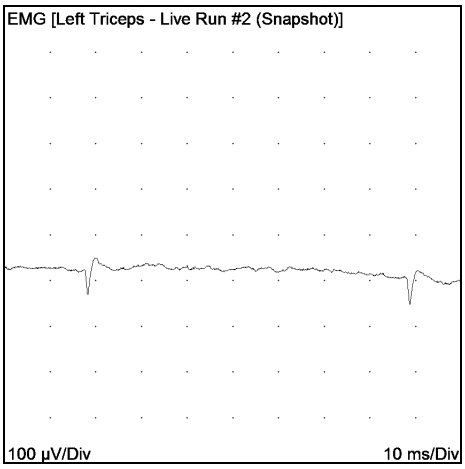

NCV [Left Ulnar Motor]
P
Wrist 8cm :
O
P
B Elbow :
O
P
A Elbow :
O
5000 µV/Div
2 ms/Div

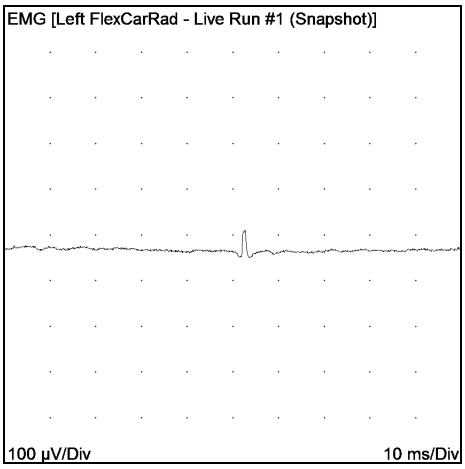

NCV [Left Median/Radial Comparison]
Median 10cm :
P
O
P
Radial 10 cm :
O
10 µV/Div
2 ms/Div

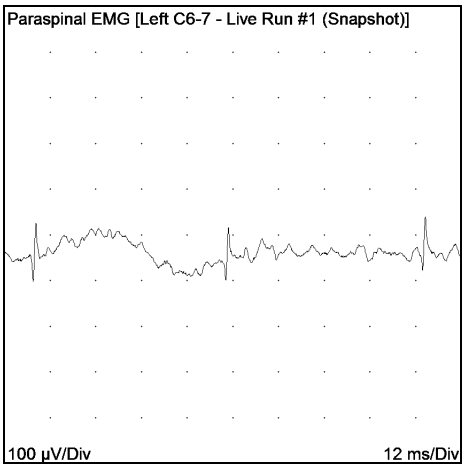

FWave [Left Median (Mrkrs)]
Tr 1
5000 µV/Div
500 µV/Div
5 ms/Div

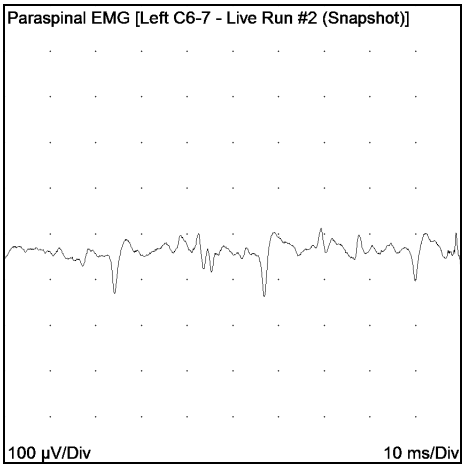

FWave [Left Ulnar (Mrkrs)]
Tr 3
5000 µV/Div
500 µV/Div
5 ms/Div


EMG [Left Triceps - Live Run #1 (Snapshot)]
100 µV/Div
10 ms/Div


EMG [Left Triceps - Live Run #2 (Snapshot)]
100 µV/Div
10 ms/Div


EMG [Left FlexCarRad - Live Run #1 (Snapshot)]
100 µV/Div
10 ms/Div


Paraspinal EMG [Left C6-7 - Live Run #1 (Snapshot)]
100 µV/Div
12 ms/Div


Paraspinal EMG [Left C6-7 - Live Run #2 (Snapshot)]
100 µV/Div
10 ms/Div